

Rajesh, a patient living with ASCVD + CKD, diabetes and obesity (cardio-metabolic progressive disease)

Even when protected by high-intensity lipid-lowering therapy, 1 in 2 CVD patients are still facing inflammatory risk.^{1*}



Rajesh, 69.

ASCVD + CKD, diabetes, obesity

3 years post-CV event

HbA1c: 7.2%

LDL-C: 58 mg/dL

eGFR: 42 mL/min/1.73m²

Current Tx: Intensified statin therapy, GLP-1 RAASi, Beta blockers, SGLT2i

- Rajesh quit smoking and rarely drinks, but he struggles with portion control and eats a lot of processed foods. His sleep is poor, and when doing light activity, he struggles with a tight chest and fatigue.
- He's on an intensive, modern SoC regimen, and his clinical values are near their targets. Why is he developing new symptoms?
- A colleague recently told you: **"We often overlook CV inflammation, but it independently increases CV risk and warrants the same clinical attention as traditional risk factors."**¹⁻³

Inflammatory risk can be detected using the prognostic biomarker hsCRP (≥ 2 mg/L).^{1,2,4}

- You decide to test for hsCRP. The result comes back:

2.6 mg/L.



- You check the ESC/EAS 2025 Dyslipidaemia Guidelines, which highlight that elevated hsCRP ≥ 2 mg/L is a key risk modifier in CVD.⁵
- You encourage him to further improve his diet and work on portion control. You schedule a follow-up appointment to discuss intensification of his current treatment regimen to reduce his overall CV risk.

Are you considering hsCRP when managing your patients care?



Footnotes:

*As calculated by Ridker¹, across the PROVE-IT (n=4,162)⁶, IMPROVE-IT (n=18,144)⁷, and SPIRE-1/SPIRE-2 trials (n=9,738; trial data were combined for analysis due to design similarity)⁸, patients with CVD received high-intensity statins, high-intensity statins plus ezetimibe, or high-intensity statins plus PCSK9 inhibition, respectively. Of these patients, 33% had inflammatory risk alone (hsCRP ≥ 2 mg/L), whereas 13% had a mix of inflammatory and cholesterol risk (LDL-C ≥ 70 mg/dL).¹

References:

- 1: Ridker PM. J Am Coll Cardiol 2018; 72:3320–3333.
- 2: Ridker PM et al. Lancet 2023;401(10384):1293–1301.
- 3: Zubrain R et al. Cardiol Ther 2024;13(3):465–491.
- 4: Mazhar F et al. Eur Heart J 2024; 45(44):4719–4730.
- 5: Mach F et al. Eur Heart J 2025; 0(0): 1–20.
- 6: Cannon CP et al. N Engl J Med 2004;350(15):1495–1504.
- 7: Spinier J et al. Vnitr Lek 2014;60(12):1095–1101.
- 8: Pradhan AD et al. Circulation 2018;138(2):141–149.

Abbreviations:

ACC=American College of Cardiology, ASCVD=Atherosclerotic cardiovascular disease, CKD=Chronic kidney disease, CV=Cardiovascular, eGFR=Estimated glomerular filtration rate, HFpEF=Heart failure with preserved ejection fraction, hsCRP=High-sensitivity C-reactive protein, LDL-C=Low-density lipoprotein cholesterol, LVEF=Left ventricular ejection fraction, NYHA=New York Heart Association, Tx=Treatment, SoC=Standard of care.